



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Mental Health Act 2014 – escalation of care

Reference #: FSFH-MENH-POL-0033

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital	Cockburn Health	Medical, Nursing, Allied Health, SIMS

1. Introduction

Cockburn Health is a standalone 75 bed healthcare facility, providing inpatient and outpatient services for mental health and alcohol and other drug services. While it provides an extension of Fiona Stanley Hospital Mental Health Service, Cockburn Health is a non-authorised site.

When a patient's acuity requires referral for examination by a psychiatrist using the WA Mental Health Act (MHA) 2014 and a Consultant Psychiatrist is not available on site the patient will be transferred to the place of examination documented on the Form 1A Referral Order. Where there is a delay in conveyance of the patient, it is crucial clinicians understand how to balance clinical and legal parameters to keep patients, staff and the community safe. This document outlines care of a patient at Cockburn Health waiting for transfer to the place of examination on a Form 1A Referral Order for psychiatric examination when a Consultant Psychiatrist is not on site.

2. Terminology

Authorised Hospital	Refers to a public hospital or part of a public hospital that has been authorised under the MHA 2014 to: • receive persons who are reasonably suspected to be mentally ill for examination by a psychiatrist, or • admit involuntary patients who are mentally ill.
Non-Authorised Healthcare Setting	Any healthcare setting that is not an authorised hospital.
Voluntary patient	A person who is proposed to be, or is being treated, by a mental health service voluntarily. This includes patients who have been referred under the Mental Health Act 2014 on a

The information provided in this document is based on its relevance to Fiona Stanley Fremantle Hospital Group (FSFHG) only. Due to differences in context, scope of practice and differences in service delivery, FSFHG does not make claim to its relevance or appropriateness for use within other organisations/sites.

	Form 1A for an examination by a psychiatrist.
Detention Order	When necessary for patient, staff and /or community safety, a medical practitioner or authorised mental health practitioner may complete a Form 3A Detention Order to detain a person to enable them to be taken to the authorised hospital or other place for a Form 1A examination by a psychiatrist. The Form 3A authorises detention for up to 24 hours from the time when the order is made (s.28(1) MHA).
Unpredictable behaviour	Includes verbal abuse, actual or threatened physical abuse, property damage, actual or threatened self-harm, attempting to leave or absconding, suicide attempts or suicide and intrusive or sexual behaviour.
Mental State Deterioration	An acute or gradual change in a person's mood or thinking marked by change in behaviour, cognitive function, perception or emotional state. These changes can be observed by staff, reported by the person themselves or their family or carers.
Notifiable events	Certain events affecting persons under the MHA 2014 require mental health services to notify the person's personal support person and/or an external person or body such as the Chief Psychiatrist.
Restraint	The restriction of an individual's freedom of movement by physical or mechanical means.
Detention	The act of keeping back, restraining, or withholding, either accidentally or by design a person or thing.
Seclusion	The confinement of a person who is being provided with treatment or care at an authorised hospital by leaving the person alone in a room or area from which it is not within their control to leave: Also applies to situations where the person agrees to or requests confinement but cannot leave of their own accord.
Personal support person (PSP)	Can be any of the following people who are supporting someone who is experiencing mental illness – close family member, carer, nominated person, the parent or legal guardian of a child and any guardian or enduring guardian of an adult.

3. Policy

The use of restraint, seclusion and detention may be permitted in certain circumstances in a non-authorised setting e.g. doctrine of necessity.

- Voluntary patients have the right to leave a hospital if they choose.

- All care and treatment will be guided by the principles of least restrictive, patient centred, trauma informed and culturally safe.
- Staff will engage the patient's family/significant others/PSP with the patient consent and involve them in treatment planning.
- The Aboriginal Mental Health Liaison must be consulted where culturally appropriate.

4. Procedure

4.1. Referral for assessment

- All staff have a role in recognising and responding to mental state deterioration and unpredictable behaviours and for implementing appropriate risk mitigation strategies to keep patients, visitors and staff safe.
- When early warning signs are observed or reported, members of the healthcare team will determine immediate actions to prevent further deterioration or escalation. Refer to FSFHG Anticipating and reducing the risk of unpredictable behaviours policy.
- Request early medical review of a patient where mental state deterioration is identified:
 - Determine and implement appropriate clinical interventions to prevent further deterioration.
- Where a patient's mental state continues to deteriorate a medical practitioner or authorised mental health practitioner (AMHP) must assess the patient to determine if they reasonably suspect **all** the following:
 - mental illness requires treatment.
 - because of the mental illness there is significant risk to the patient or others.
 - the patient cannot demonstrate capacity to make treatment decisions.
 - treatment in a non-authorised setting is no longer appropriate.
 - there is no alternative that would be less restrictive to provide the treatment the person needs.
- The medical practitioner or AMHP may then refer a person for an examination to be conducted by a psychiatrist by completing a Form 1A.

4.2. Consultant Psychiatrist on site

- The Form 1A will have the place of examination listed as Cockburn Health.
- If where the person is (at the time of detention) and the place of examination on the Form 1A are the same e.g., the person is at Cockburn Health, a Form 1A is completed in respect of the person and the place of examination is

Cockburn Health, then the person is taken to have been received and the use of reasonable force is permitted in order to detain the person.

4.3. Consultant Psychiatrist not on site

- Request the patient's consent or that of the next of kin to remain at Cockburn Health until transport to the place of examination is arranged and available.
- When a patient referred under a Form 1A for assessment attempts to leave they may **only** be prevented from leaving the ward when **all** the following apply:
 - there is clear and imminent risk to the safety of the patient and/or harm to others and
 - staff reasonably believe all other safe options have been exhausted and an immediate clinical decision to detain the patient is necessary and appropriate to prevent serious harm to the patient or others and
 - The person lacks capacity, or the person is of uncertain capacity and is experiencing an episode of acute behavioural disturbance requiring urgent assessment and
 - staff can provide continuous 1:1 monitoring of the patient while waiting for a bed or transport.

Persons detained at Cockburn Health must have a completed Form 1A Referral for examination by a psychiatrist and a Form 3A Detention Order, including the Checklist of requirements contained related to the form.

These forms do not provide authority to restrain or provide emergency treatment to a person in a non-authorised setting.

- The Form 1A must specify the place where the referred person is to be examined. Document the place of detention as Cockburn Health on the Form 3A.
- Complete Form 4A Transport Order for transport of the patient to the place of examination.
- Clinical staff will ensure all the principles relating to detention s.170 MHA 2014 are applied including:
 - the person is detained for a brief a period as practicable.
 - degree of force used must be minimal.
 - least possible restriction on freedom of choice and movement

- patient privacy is respected, and they are treated with dignity and respect.

In the absence of authority, a person can only continue to be detained/restrained in a non-authorised setting for as long as the risk remains imminent to avoid irreparable damage to the person or others.

- The person detained must have the least possible restrictions consistent with the person's detention and be always treated with respect and dignity.

4.4. Patient monitoring

- The decision to detain a patient must be made collaboratively by the Treating or On Call Consultant and senior clinicians on site based on the presence of clear and imminent risk in the patient's presentation.
- The specific rationale and timeframe for review of removing the patient's ability to leave the ward will be documented contemporaneously in the patient's digital medical record (DMR) with all relevant details.
- The Treating or On Call Consultant will determine the level of patient observation required. A minimum of 1:1 Level 2 observations (within eyesight) must be maintained. Refer to the FSFHG Patient Observation – Specialising and Close Observations policy and procedure.
- The patient will be nursed in an appropriate area and visual patient observations will be completed by an appropriately qualified and skilled clinician as ordered.
- While on 1:1 special, the clinician will document on the Patient Observation Chart contemporaneously every 15 minutes including:
 - Patient presentation
 - Interactions
 - Medications
 - Level of agitation/risk related behaviours
- Update the DMR once period of observation has ended.
- For patients exhibiting levels of aggression and risk to others, security staff will be requested to be present in the area, where they can offer assistance/support in approaching the patient for medication administration, refreshment, toilet access if required.
- Clinical staff are the decision maker and will provide direction to security staff who may offer supplementary assistance when directed.

4.5. Patient review

- There is an ongoing obligation to ensure any detention of a person is used only when necessary and at an absolute minimum for the purpose.
- Immediate review of the patient if their condition deteriorates for any reason including injury.
- If the patient's presentation changes, risk appears to have decreased and the patient becomes agreeable to waiting for transport, the Treating Consultant or on call consultant will be requested to immediately review the patient to allow them to no longer be detained. The rationale to be documented in the DMR.
- An increase in observation levels (to within arm's length) can be initiated at any time when clinically indicated. Rationale to be documented on DMR.

5. Compliance/Performance Monitoring

The Cockburn Health Nurse Director will ensure:

- data is collected and monitored on the use of detention including numbers, circumstances of events, patient characteristics, and outcomes.
- the data will be monitored by the Cockburn Health Safety Quality Risk Committee quarterly.
- a Datix CIMS is completed if the detention of the patient results in actual or potential patient harm.
- incidents will be reviewed at the Incident Review Meeting attended by medical, nursing and allied Health staff. Review captured on MR671 Form.
- Service 5 Safety Quality Risk Committee will routinely monitor clinical incident trends related to behaviour.

6. Related Policy Documents

WA Health:

- [Use of Restrictive Practices in Non – Authorised Healthcare Settings](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [Principles and best practice for the care of people who may be at risk of exhibiting violent or aggressive behaviour](#)
- [Principles and best practice for the care of people who may be suicidal](#)
- [Requesting Road based transport for mental health patients subject to Transport Orders](#)

- [Clinical Incident Management Policy](#)

SMHS:

- [Rights of patients in mental health SMHS:164](#)
- [Notifications in Mental Health SMHS:165](#)

FSFHG:

- Clinical Handover (FSFH-HW-POL-0035)
- Clinical Documentation (FSFH-HW-POL-0039)
- Clinical Risk Assessment and Management (FSFH-HW-POL-0017)
- Anticipating and reducing the risk of unpredictable behaviours (FSFH-MENH-POL-0027)
- Patient Observation – Specialising and Close Observations (FSFH-MENH-POL-0023)

7. Related Documents

[The Mental Health Act 2014](#)

[WA OCP Guideline Use of Detention and Restraint in Non-Authorised Healthcare Settings](#)

8. Related Standards

NSQHS Standards:

- Clinical Governance
- Partnering with Consumers
- Comprehensive Care
- Communicating for Safety
- Recognising and Responding to Acute Deterioration

Chief Psychiatrists Standards for Clinical Care:

- Risk assessment and management
- Seclusion and bodily restraint reduction

9. Authorisation

EXECUTIVE SPONSOR: Service 5 Nurse Director					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	08/2024	FSH CH Nurse Director	OCP, Legal	Service 5 Safety Quality and Risk Committee	11/2027